

RxKids Hawai'i - Cost & Benefits by Income Quintile, TY 2028

Statutory eligibility: Medicaid (clause 1) OR income <= 300% FPL incl. expected unborn child (clause 2)
Point estimate - true uncertainty is +/-30-40%. No admin caseload exists to calibrate against; cost hinges on soft pregnancy/infant incidence assumptions. Read with the assumption band.

Fiscal cost (annual)

Total program cost (benefit)	\$32,058,217
Prenatal arm	\$10,100,534
Postnatal arm	\$21,957,683
Administrative load (8%)	\$2,564,657
Appropriation total (benefit + admin)	\$34,622,875
Sampling 90% CI	\$24,364,840 - \$39,751,595
Assumption band	\$19,234,930 - \$40,874,837

Scenario comparison

Take-up held constant (0.90 newborn / 0.83 prenatal); only the eligibility gate and postnatal window change. Cost = annual benefit dollars; appropriation adds the admin load.

Statutory (Medicaid OR <=300% FPL) · 6-mo postnatal	\$4,500	\$32,058,217	\$34,622,875	14,053
Universal · 6-mo postnatal	\$4,500	\$53,214,714	\$57,471,892	23,327
Universal · 12-mo postnatal	\$7,500	\$89,663,149	\$96,836,201	23,327

Potential option - extend postnatal 6-12 months

The program may opt to extend postnatal payments to the full 12 months (Flint's upper bound). Priced here as an optional add-on:

Additional cost (+6 months)	\$21,957,683
Total cost (12-month design)	\$54,015,901

First fiscal year (launch) - 12mo operating, 12mo enrollment ramp

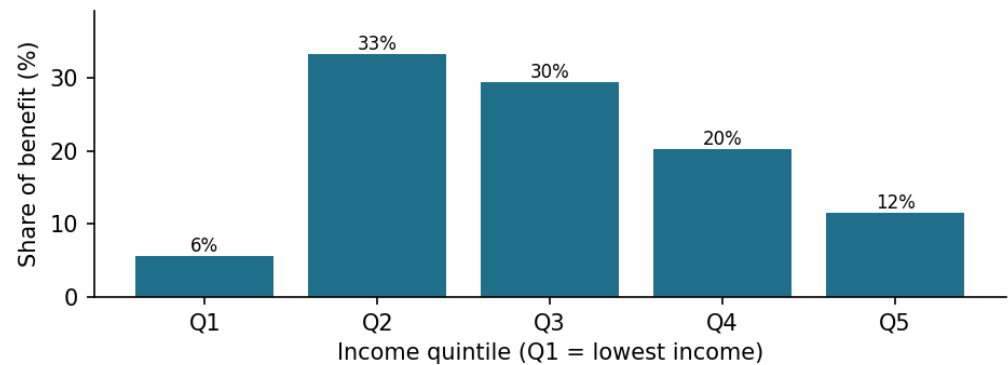
Year-1 cash disbursement, below steady state: enrollment ramps up and the 6-month postnatal caseload takes time to fill.

Year-1 disbursement (total)	\$13,298,630
Prenatal / Postnatal	\$5,471,123 / \$7,827,507
Postnatal deferred to next FY	\$4,066,238
Year-1 as % of steady state	41%
Ramp sensitivity (6 / 12 / 18 mo)	\$20,804,905 / \$13,298,630 / \$8,865,753

Household reach

Eligible families (weighted)	7,340
Expected recipients / year	14,053
Expected pregnancies / infants	6,734 / 7,319
Avg benefit per recipient	\$2,281

Benefit received by income quintile



Quintile	Avg income	Families	Recipients	Total benefit	Share
Q1	\$17,179	110,267	776	\$1,770,843	5.5%
Q2	\$57,026	110,263	4,685	\$10,687,449	33.3%
Q3	\$96,722	110,317	4,143	\$9,450,131	29.5%
Q4	\$154,809	110,231	2,836	\$6,469,878	20.2%
Q5	\$334,087	110,343	1,613	\$3,679,916	11.5%

Cost by county

County	Cost total	Prenatal	Postnatal	Recipients
Hawaii	\$6,932,610	\$2,184,247	\$4,748,363	3,039
Honolulu	\$21,175,095	\$6,671,605	\$14,503,490	9,282
Kauai	\$2,288,733	\$721,108	\$1,567,625	1,003
Maui	\$1,661,779	\$523,574	\$1,138,205	728

Both arms are driven by OBSERVED births (age-0 dependents in each PUMS tax unit), calibrated to CDC NVSR resident births and projected to the target year with the repo's damped-trend ensemble. Each eligible birth draws one prenatal + one postnatal payment. Cost = the take-up-adjusted benefit weighted by the household (WGTP) weight. Eligibility is tested on MAGI at the tax-unit (MAGI-household) grain, the family Medicaid uses. Benefits-by-quintile rank SPM units on summed income into population-equal fifths. Sampling CI is ACS sampling error (SDR, 80 replicate weights); the assumption band sweeps take-up and the birth count (+/-25%). 2026-2028 FPL is CPI-projected off the 2025 HHS table. Full methodology: RXKIDS_METHODODOLOGY.md.